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Name: جيهان حسين ابراهيم خليل National ID: 27511192301382	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
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المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Supportive and palliative Treatment
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التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

مجلس الوزراء
مملكة البحرين



جهان
حسين ابراهيم خليل
٢٩
ش فرحت
الجزيرة - البحرين



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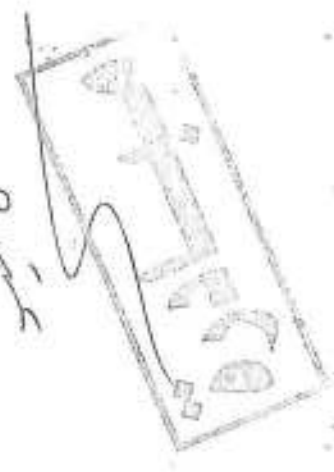


٢٠١٩/١٢ ٢٧٥١١١٩٢٣٠١٣٨٢
رية منزل

مكتوبة
مسلمة
أش
محمد عبدالواحد محمد
٢٠٢٦/١٢/٠٩
البطاقة سارية حتى



فصل ملف لدى
١٨ منال عيسى



د. يوسف روستا

دكتور في الطب

مبادرة رئيس الجمهورية

لدعم صحة المرأة المصرية

الصحة العامة

مصر

08/04/2026

MDT FORM

Name:	Jihan Huseen Ibrahim .	MRN:	1384
Age:	51 Y	ID:	27511192301382
Menopausal Sta		Phone:*	01100264411
NRS SCORE:		Marital Status:	
BMI:		Family History:	
Gender		Reason Of MDT:	Metastatic
CO Morbidity:		ECOG Performance Status	0 0 1 2 3 4
Surgical History:*	BCS		

Clinical Data:

BX:

IDC

ER:

0/8

PR:

6/8

HER2:

3+

KI67:

History:

female Pt 50 yrs pre HTN +ve FH her mother thyroid cancer >> RT BC T2 N0 >>> ER 0/8 PR 6/8 her2+3 IDC >>> upfront BCS (12-2024) + SLNB >>> adj Cth AC *4 TXT *4 >>PORT >> Pt didnot receive anti her2 >> on tam since 6-2024 till now O/E>> Rt inflammatory breast with rigidity BSM 5-2026 >>> Lt axillary LNs about 1.2*0.8 cm , extensive infiltrative involving rt breast BSM 18-6-2026 >> Rt extensive inflammatory carcinomatosis of rt breast
O/E: Rt breast locally advanced disease + skin nodules on abd wall
Complaining from cough and dyspea - impending visceral crisis

Sonammammography:

Biomarker/Molecular

Pending cytology from effusion

Mets Workup:

CT CAP: new LT Axillary LN + Rt sided marked pleural effusion + Newly developed osteolytic bony lesions.
Pending Bone scan

Recommendations:

Navelbine + Herceptin + Zometa

MDT Outcome & Plan

Navelbine + Herceptin + Zometa

Right Side				Left Side			
Q1	Q2	Q3	Q4	Q1	Q2	Q3	Q4

Clinical Staging:

Pathological Staging:

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
د خالد عبد العزيز	د محمد نبراوي		

MDT Form
[[DOCTYPE:MDT_FORM]]

Name: <u>جيهان حسين ابراهيم</u>		
MR. : <u>1384</u>		
Detailed Medical Report		
		السيد
تحتية طبية وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
ننشرف بأن نوضح فيما يلي تقريراً عن حالته		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد انه		
The patient is a 50-year-old individual with a past medical history of hypertension and a positive family history of thyroid cancer in the mother. The clinical course began with a right-sided breast cancer classified as T2 N0, for which the patient underwent upfront breast-conserving surgery in December 2024 with sentinel lymph node biopsy, followed by adjuvant cyclophosphamide-based chemotherapy (four cycles) and radiotherapy. Anti-HER2 therapy was not administered at that time, and tamoxifen was started in June 2024 and continued to the present. On examination in May 2026, the patient presented with a right inflammatory breast exhibiting rigidity, skin nodules on the abdominal wall, and left axillary lymph nodes measuring approximately 1.2 × 0.8 cm, with extensive infiltrative disease of the right breast noted on June 18 2026. The patient reports cough and dyspnea, suggestive of an impending visceral crisis. Histopathology confirmed invasive ductal carcinoma (IDC) with an estrogen-receptor score of 0/8, progesterone-receptor score of 6/8, and HER2 3+ positivity. Imaging demonstrated a right-sided marked pleural effusion, new left axillary lymphadenopathy, and newly developed osteolytic bone lesions on CT of the chest, abdomen, and pelvis; a bone scan is pending. These findings are consistent with locally advanced disease with cutaneous involvement and distant metastases. The referring team recommended treatment with Navelbine, Herceptin, and Zometa. The multidisciplinary team concurred and the final plan is to proceed with Navelbine, Herceptin, and Zometa.		
تحريراً في ,		
	التاريخ : 23-09-2026	الوقت :
		الطبيب المعالج :
يعتمد ,		



Patient Name	: الأستاذ / جيهان حسين ابراهيم خليل	Request No.	: 13005914
Patient No.	: 27511192301382	Request Date	: 11/09/2023
Path No.	: SR-1040/23	Verification Date	: 28/09/2023
Sex	: Female	Financial Category	: اورام دار السلام - اقتصادي
Age	: 47 Y	Department	: Out Patient
Referring Doc :		Registered by	: .marwa mohamed youss

Pathology Report

Clinical data
Right breast mass

Nature of specimen
Revision of slides
Gross examination
Two blocks (200779) (203291 + 6 slides.

Microscopic examination
* Histological type: Invasive duct carcinoma
* Grade (differentiation +nuclear pleomorphism+mitosis): (3+3+2) Grade III
* DCIS: No
* Angioinvasion: Could not be detected within the limits of biopsy received
IPT :

ER:No nuclear staining in tumor cells(0/8)
PR:strong nuclear staining in 40% of tumor cells (6/8)
Her2: Strong complete membrane staining in >10 of tumor cell , score(+3)
*Cytology&cell block: positive for malignant cells

Diagnosis
Right breast mass ; slide revision of tru cut biopsy specimen :
invasive duct carcinoma, Grade III.

IPT :
ER: Negative (0/8)
PR:Positive(6/8)
Her2: Positive(+3)
Cytology&cell block: positive for malignant cells
Diagnosis

Signature

Verified by :
Dr.Heba Hasan



Presidential Initiative for Women's Health - Pathology Lab
المبادرة الرئاسية لصحة المرأة - معمل الباثولوجي

Patient Name	: الأستاذة / جيهان حسين ابراهيم خليل	Request No.	: 13005914
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Age	: 47 Y	Department	: Out Patient
Referreing Doc :		Registered by	: .marwa mohamed youss

Pathology Report

Prof. Dr. Amal Hareedy
Dr. Heba Mostafa

Verified by :
Dr. Heba Hasan

Printing Date : 30/09/2023 09:48:30

Page 2 of 2

Printing By : .marwa mohamed youss



Patient Name	: الأستاذة / جيهان حسين ابراهيم خليل	Request No.	: 13008872
Patient No.	: 27511192301382	Request Date	: 05/12/2023
Path No.	: 4324/23	Verification Date	: 25/12/2023
Sex	: Female	Financial Category	: اورام دار السلام - اقتصادي
Age	: 48 Y	Department	: Out Patient
Referring Doc	:	Registered by	: marwa mohamed youss

Pathology Report

Clinical Data: Right breast cancer, Up-Front.

Nature Specimen: Right breast conservative surgery , sentinel lymph nodes and axillary clearance.

Gross Examination:

- * Procedure: Right breast conservative surgery
- Fibrofatty tissue piece measured 11.5x6x3 cm with covering skin ellipse measured 10x3 cm.
- * Tumor extent: A wired greyish white firm tumor .
- * Size: 1.8x1x1 cm. 11
- * Distance from margins: 0.3 cm away from inferior margin (least margin).
- * Rest of the breast showed greyish white areas with dilated ducts.
- * Skin involvement: Unremarkable grossly
- * Sentinel lymph nodes: Fibrofatty tissue piece measuring 3.5x2.5x1 cm entangling 10 lymph nodes.
- * Axillary clearance : Fibrofatty tissue measured 8x5x2 cm entangling 20 lymph nodes .

Microscopic Examination:

- * Histological type: Ivasive duct carcinoma .
- * Grade (differentiation+ nuclear pleomorphism+ mitosis):(3+2+1) Grade II.
- * Ductal carcinoma in situ: Yes (high grade comedo)
- * Resection margins: Free margins .
- * Microcalcifications: Yes.
- * Additional pathologic findings: Surrounding breast showed fibroadenosis .
- * Lymph node involvement: Positive lymph nodes (16/30)
- * Frozen section : Positive sentinel lymph nodes (5/5) and free margins .

Diagnosis:

Verified by :
Rana fathy





Patient Name	: الأستاذ / جيهان حسين ابراهيم خليل	Request No.	: 13008872
Patient No.	: 27511192301382	Request Date	: 05/12/2023
Path No.	: 4324/23	Verification Date	: 25/12/2023
Sex	: Female	Financial Category	: اورام دار السلام - اقتصادي
Age	: 48 Y	Department	: Out Patient
Referreing Doc :		Registered by	: .marwa mohamed youss

Pathology Report

Right breast conservative surgery , sentinal lymph nodes and axillary clearance , Up-front:
Invasive duct carcinoma , grade II with DCIS .
- Free margins .
- Stage: pT1c, pN3

Prof. Dr. Gamal Abdelaty
Dr. Marwa Mahmoud

Verified by :
Rana fathy



Patient Name	جيهان حسين ابراهيم	Case No.	344848
Sex/Age	Female 50 Y	Patient No.	25323
Registered Date	29/7/2026 12:44:19 PM	Referred By Dr.	Interventional Radiology
Validation Date	22/8/2026 02:32:11 PM	Branch	Gustave Roussy
Reporting Date	22/8/2026 03:19:28 PM	Pathology No.	C-167/26

Cytology Report

Clinical Data: Right sided pleural effusion in a patient with history of right MRM.

Nature of specimen: Right pleural effusion, Cytology, collected on 29/07/2026.

Gross examination:

Four alcohol fixed (H&E) stained smears & one cell block prepared from 20 cc of yellow fluid.

Technique:

- Sections prepared from paraffin block for immune-staining using Benchmark ULTRA system for;
SOX10 clone (SP267) & GATA3 clone (L50-823).

Microscopic examination:

Cytological examination of smears & cell- block prepared reveals cellular smears entangling malignant cells arranged in small cohesive clusters & papillary fronds.

Results:

*** IPT (All controls showed appropriate reactivity):**

- SOX10: Negative staining in tumor cells.
- GATA 3: Negative staining in tumor cells



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة 1 - مصر
 Room No. (4) - First Floor-Building No. 987- Nile Corniche - Mizr El Qadima - Cairo, Egypt

Patient Name	جيهان حسين ابراهيم	Case No.	344848
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Validation Date	22/8/2026 02:32:11 PM	Branch	Gustave Roussy
Reporting Date	22/8/2026 03:19:28 PM	Pathology No.	C-167/26

Diagnosis:

Right sided pleural effusion, Cytology:

- Positive for malignant cells.
- Further immunohistochemical studies for (MOC31, Calretinin & CK) are required to verify their exact origin.

Signature

Prof. Dr. Maha Guimei.



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر الجديدة - القاهرة - مصر
 Room No. (4) - First Floor - Building No. 987 - Nile Corniche - Mide El Qadima - Cairo, Egypt

Name : Gihan Hussein Ibrahim Khalil

Date : 15-08-2026 07:01

Branch : Dokki

Medical No. : 439467

Ref.Physician Prof : Goustav Roussey Egypt

R Tc-99m MDP SKELETAL SCINTIGRAPHY

E History:

51-year-old female patient with history of surgically treated right breast cancer, received chemo-radiotherapy and hormonal treatment, referred for skeletal survey.

E Technique:

Anterior and posterior whole-body scans 2 hours post injection of 20 mCi of the tracer.

C Findings :

- Considering the last bone scan report dated 21/12/2024, the current scan revealed newly developed multiple, randomly distributed foci of variable tracer uptake keeping with bone secondaries. They are located at the sternum, around both shoulders, proximal parts of both humeri, dorso-lumbar vertebrae, bilateral scattered ribs, pelvic bones, proximal and distal parts of both femora as well as proximal parts of both tibiae.
- Osteoarthritic features are noted around big joints.

C IMPRESSION:

- Multiple active osseous lesions at the aforementioned sites most likely metastatic.
- Follow up bone scan is advisable to monitor response to therapy.

Much obliged
Dr. Alaa Nagy, MD
Nuclear medicine

Patient's Name: Gihan Hussein Ibrahim Khalil
Date: 2026-07-14
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt



MULTISLICE CT EXAMINATIUNON OF THE CHEST ,ABDOMEN AND PELVIS WITH CONTRAST ADMINISTRATION REVEALED:-

- Evident right MRM with no evidences of either residual/recurrent neoplastic lesions seen at the operative bed. The overlying skin is till diffusely thickened, reaching to about 10mm in the peri-areolar region and few retro-pectineal dense lymph nodes are also still seen with the largest one measures about 12mm.
- Normal left breast.
- Newly-developed multiple variable sized left axillary lymph nodes are seen with loss of hilar fat contents in some of them with the largest one measures about 16mm.
- Marked right sided pleural effusion with retraction of the underlying lung towards the hilum.
- Newly-developed ground-glass densities are seen in the anterior segment of right upper pulmonary lobe and a patchy area of consolidation seen in the middle lobe and in the anterior segment of right lower pulmonary lobe.
- No sizable abnormally-enhancing pulmonary nodules ,other patchy areas of consolidations or calcifications noted.
- No sizable abnormally-enhancing mediastinal or hilar lymph nodes seen.
- Patent central airways.
- No left pleural or pericardial effusion.
- Average size of the heart with normal CT of its big vessels.
- Newly-developed foci of osteosclerotic bony lesions are seen widely scattering in the scanned skeleton.
- Average sized liver, measuring about 14.5 cm, showing a non-homogenous parenchymal attenuation and enhancement pattern with evidences of patchy low density seen in its right lobe.
- No sizable abnormally-enhancing focal lesions or dilated biliary radicles.
- Patent PV.
- Average sized spleen, measuring about 8.8 cm. No focal lesions seen inside.
- Normal CT appearance of the pancreas, functioning both kidneys, both suprarenal gland, aorta and IVC.
- No sizable abdominal or pelvic lymphadenopathy.
- No ascites.
- No intra-vesical stones, solid masses, wall thickening or diverticulosis seen.
- The rest of the pelvic structures are unremarkable.



Patient's Name: Gihan Hussein Ibrahim Khalil
Date: 2026-07-14
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

IMPRESSIONS:

- Evident right MRM with no evidences of either residual/recurrent neoplastic lesions seen at the operative bed. The overlying skin is till diffusely thickened, and few right retro-pectineal dense lymph nodes are also still seen.
- Newly-developed multiple variable sized left axillary lymphadenopathy.
- Marked right sided pleural effusion with retraction of the underlying lung towards the hilum,denoting a progressive course.
- Newly-developed ground-glass densities in the anterior segment of right upper pulmonary lobe and a patchy area of consolidation also seen in the middle lobe and in the anterior segment of right lower pulmonary lobe.
- Newly developed foci of metastatic osteosclerotic bony lesions widely scattering in the scanned skeleton.

Prof/Hanan El-Ahwal.

Name Jihan ibraheem Hussein khaleel

Chest X-Ray Revealed

Lung fields : obliteration of the right cpa likely pleural effusion

Enlarged hilar bronchovascular markings and Mildly enlarged Cardio- mediastinal silhouette

Diaphragm : hemidiaphragms appear normal in contour

Bones and soft tissue : no acute osseus abnormality identified

ECHO REPORT

NAME: حسان

DATE: 23/8/2026

ID NUMBER: 1

SEX: AGE:

EF	<u>62%</u>	55-75%	PWT	<u>0.8</u>	0.6-1.1cm
LVEDD	<u>4.3</u>	3.5-5.8cm	AO.R	<u>2.3</u>	3-3.7cm
LVESD	<u>2.7</u>	2-4cm	LA	<u>3.6</u>	2-4cm
FS	<u>28%</u>	25-45%	RV		0.7-2.3com(supine)
SWT	<u>0.9</u>	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV Dimensions Normal

LV Wall Thickness Normal

LV Diastolic Function GE

SWMA No. RWMA

LA and Ao Root Dimensions Normal

Mitral Valve Mild MR

Aortic Valve N

Tricuspid Valve Trivial TR

Pulmonary Valve

Right Side and Systolic Function TAPSE: 2.1

Intracardiac Masses and Effusion

DR. محمد



Exam Information:

<u>Patient Name:</u> GEHAN Hussain	<u>Date of birth:</u> 19/11/1975
<u>Arabic Name:</u> جيهان حسين	<u>Patient Gender:</u> F
<u>Patient RIS ID:</u> 336652	<u>Exam Date:</u> 24/10/2023
<u>Patient HIS ID:</u> 123018845	
<u>Written by:</u> Dr. Hoda Fathy	<u>Approved by:</u> Dr. Hoda Fathy

Nuclear Medicine Unit

وحدة الطب النووي

^{99m}Tc -MDP BONE SCAN

Brief history: 48 years old female patient diagnosed with managed breast cancer.

Desired Information: - skeletal survey.

Technique:

Delayed Whole body imaging in anterior and posterior projection performed 2 hours after I.V administration of 20 mCi ^{99m}Tc -MDP in the following manner:

Report:

- ☐ No hot spot that account for osseous deposits could be detected allover the surveyed skeleton.
- ☐ Spondylo-degenerative changes are seen over dorso-lumbar vertebrae.
- ☐ Pronounced osteoarthritis noted over both knee joints.
- ☐ Both kidneys are normally visualized.

Conclusion:

Negative study for the detection of any definite metastatic osseous deposits.

Patient Name	جهان حسين ابراهيم	Case No.	358888
Sex/Age	Female 50 Y	Patient No.	26042
Registered Date	18/8/2026 12:21:05 PM	Referred By Dr.	Out Patient
Validation Date	13/9/2026 02:35:20 PM	Branch	Gustave Roussy
Reporting Date	13/9/2026 02:58:25 PM	Pathology No.	SR-963/26

Pathology Report

Clinical Data: Known Case Of Breast Cancer Presented With Right Upper Abdominal Skin Nodule.

Nature of Specimen: Block Revision.

Gross Examination:

- One paraffin block labelled by (716/26) was received.

Technique:

- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for;
 - GATA-3, ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic examination:

- Histopathological examination of sections prepared revealed skin biopsy with dermal deposits of malignant cells disposed in clusters and individual cells of cohesive neoplasm with enlarged hyperchromatic nuclei.

Results:

*** IPT (All controls showed appropriate reactivity):**

- GATA-3: Positive nuclear reaction in tumor cells.
- ER: Weak nuclear reaction in 10% of tumor cells (score 3/8).
- PR: Moderate nuclear reaction in 5% of tumor cells (score 4/8).
- Her2: Strong complete membrane reaction in more than 10% of tumor cells (score 3+).
- Ki67%: 80%.

Conclusion:

Known Case Of Breast Cancer With Abdominal Skin Nodule; Punch Biopsy Block Revision:

- Skin Dermal Deposits Of Breast Origin.

IPT:

- ER: Positive (score 3/8).
- PR: Positive (score 4/8).
- Her2: Positive (score 3+).
- Ki67%: 80%.

****Scoring system: "ER, PR"**

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ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

**** Scoring system: "Her2"**

Reference: ASCO-CAP Her2 Test Guideline Recommendation. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of

the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

SIGNATURE

Dr. Walaa Ibrahim Ghonaim. MD.

Patient Name	جهان حسين ابراهيم	Case No.	358888
Sex/Age	Female 50 Y	Patient No.	26042
Registered Date	18/8/2026 12:21:05 PM	Referred By Dr.	Out Patient
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Reporting Date	24/9/2026 01:31:47 PM	Pathology No.	SR-963/26

Pathology Report

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Technique:

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Microscopic examination:

- Histopathological examination of sections prepared revealed skin biopsy with dermal deposits of malignant cells disposed in clusters and individual cells of cohesive neoplasm with enlarged hyperchromatic nuclei.

Results:

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- Her2: Strong complete membrane reaction in more than 10% of tumor cells (score 3+).
- Ki67%: 80%.

Conclusion:

Known Case Of Breast Cancer With Abdominal Skin Nodule; Punch Biopsy Block Revision:

- Skin Dermal Deposits Of Breast Origin.

IPT:

- ER: Positive (score 3/8).
- PR: Positive (score 4/8).
- Her2: Positive (score 3+).
- Ki67%: 80%.

****Scoring system: "ER, PR"**

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ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, > 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

**** Scoring system: "Her2"**

Reference: ASCO-CAP Her2 Test Guideline Recommendation. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of

the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

SIGNATURE

Dr. Walaa Ibrahim Ghonaim. MD.